



File No.: EXP202207561

RESOLUTION OF RIGHTS PROCEDURE

After carrying out the procedural actions provided for in Title VIII of Organic Law 3/2018, of December 5, on the Protection of Personal Data and the Guarantee of Digital Rights (hereinafter LOPDGDD), the following FACTS have been established:

FIRST: On July 6, 2022, a complaint was received by this Agency filed by A.A.A. (hereinafter, the complainant) against CLÍNICA TORCALDENT, S.L. (hereinafter, the respondent) for considering that their right of access to their medical history had not been duly attended to.

The complainant states that they requested a copy of their medical history and that it was provided incomplete and untruthful. It is indicated that the following documentation is missing: X-rays, photographs, and molds. Furthermore, they question its content and completion, as well as the medical treatment carried out.

The respondent indicated in their response to the complaint transfer procedure carried out by this Agency that the complaint is due, more to discrepancies between the parties and their alleged dissatisfaction with the treatment and healthcare service provided, rather than to any problem in attending to the requested right of access. A copy of the burofax dated July 22, 2022, is provided, in which the supplementary documentation to the previously delivered medical history is made available. Therefore, it was considered that the right had been attended to.

SECOND: Dissatisfied with the resolution, the complainant filed a motion for reconsideration arguing that, despite having received a response after the complaint was filed, the supplementary information is lacking in completeness since X-rays, photographs, and molds have still not been delivered.

The corresponding Reconsideration Procedure was instructed, in which the respondent was given the opportunity to be heard. The Director of this Agency deemed the motion well-founded because evidence was provided that the complete medical history had not been made available.

THIRD: As a consequence of the acceptance of the aforementioned Motion for Reconsideration, the present Rights Procedure was opened, transferring the documentation to the respondent, so that within ten working days they could present the allegations they deemed appropriate, stating, in summary, that "(...) My represented party, as has been stated at all times during the procedure in question, has delivered the complete copy of the patient's medical history (as the patient stated, "due to the volume of documentation" it was signed without checking it, when it could have been done right there) but, furthermore, shortly thereafter, upon checking that some document had been left pending delivery, on June 22, the rest was made available to them.

As you can understand, the clinic I manage cannot jeopardize its professional reputation over an absurd matter (with all due respect) of alleged omission regarding the delivery of documents. The entirety of what was available has been delivered, and there is no document that has not been delivered because we are aware that it does not belong to us and that it is of their legitimate property.

THIRD: After having presented the previous allegations, I proceed to reiterate what was stated by the Agency itself in the initial resolution, and it is nothing but that what is questioned here is not any problem in attending to the requested right of access, but rather discrepancies between the parties and an alleged dissatisfaction with the treatment and healthcare service provided, and that such discrepancy should be resolved in other forums and not in the one we are in, although we are also very calm in the other aspect since, at all times, as can be clearly observed in the administrative file itself, the patient was informed of all medical acts carried out and all protocols required for good practice were followed, as could not be otherwise. (...)"

FOURTH: After examining the document presented by the respondent, it is transferred to the complainant, so that, within ten working days, they may formulate the allegations they consider appropriate, indicating, in summary, and as far as this procedure is concerned, that

"I have only been provided with:

1) 2 sheets, sent by burofax on 07/27/2022, which are copies of X-rays, in which can be seen:

- The teeth before starting treatment and
- The teeth after receiving treatment, with the implant in 1, which was placed by the Dr. (...) and the implant in 12 that was placed so that they would be individual; in the paper X-ray, it can be seen that 12 is stuck to 11. (The other structures of the face are not visible, nor the roots of the teeth, nor gums...) that is NOT X-RAYS.

2) The medical history, which I sent to the AEPD in July 2022. It is not a COMPLETE Medical History. By the way it is written, I am impressed that it was written by B.B.B., just like the response to my first complaint to the clinic, on 01/22/2021, which is signed by the Torcaldent team, as it was sent from the phone of B.B.B. Everything that was written that day was false; he did not know me, nor had he ever seen me. (...)"

They say they have destroyed my documents. What program do they have that History in? If it is a Word or a specific program, it can be recovered.

The data protection officer must know this. It is strange that in all these months, they have not accessed the computer system and retrieved my History and compared it with the one I sent. I thought they were taking so long to respond because they were investigating. And my mold, which they have never let me see. Shouldn't they keep it for 5 years? Wasn't it their tool?"

FIFTH: The responsible party has been granted the opportunity to be heard, reiterating their previous allegations.

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"As has been repeatedly stated, everything requested by my client was provided to her and, in any case, the entirety of the medical history was delivered. In any event, some photographs or imaging evidence were not included (and were evidently destroyed) because, in the opinion of the professionals responsible for her treatment at each moment, they were not deemed relevant to the course of the medical treatment being followed."

LEGAL GROUNDS

I

Competence

In accordance with the powers granted to each supervisory authority by Article 58.2 of Regulation (EU) 2016/679 (General Data Protection Regulation, hereinafter GDPR), and as established in Articles 47, 48.1, and 64.1 of Organic Law 3/2018, of December 5, on the Protection of Personal Data and Guarantee of Digital Rights (hereinafter, LOPDGDD), the Director of the Spanish Agency for Data Protection is competent to initiate and resolve this procedure.

Furthermore, Article 63.2 of the LOPDGDD states that: "The procedures processed by the Spanish Agency for Data Protection shall be governed by the provisions of Regulation (EU) 2016/679, by this organic law, by the regulatory provisions issued in its development, and, in so far as they do not contradict them, subsidiarily by the general rules on administrative procedures."

II

Preliminary Issues

In accordance with the provisions of Article 55 of the GDPR, the Spanish Agency for Data Protection is competent to perform the functions assigned to it in Article 57, including the enforcement of the Regulation and promoting awareness among data controllers and processors regarding their obligations, as well as addressing complaints filed by a data subject and investigating, as appropriate, the reasons for such complaints.

Correspondingly, Article 31 of the GDPR establishes the obligation of data controllers and processors to cooperate with the supervisory authority that requests it in the performance of its functions. In cases where they have appointed a data protection officer, Article 39 of the GDPR assigns this officer the function of cooperating with that authority.

In accordance with this regulation, following the procedure established in Article 65.4 of the

LOPDGDD, prior to the admission of the complaint regarding the exercise of the rights regulated in Articles 15 to 22 of the GDPR, which gives rise to this procedure, the complaint was forwarded to the respondent party for analysis, requiring them to respond to this Agency within one month and to demonstrate that they had provided the complainant with the appropriate response.

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The admission of the complaint determines the initiation of the present procedure for the lack of attention to a request for the exercise of the rights established in Articles 15 to 22 of the GDPR, regulated in Article 64.1 of the LOPDGDD, which states:

"When the procedure refers exclusively to the lack of attention to a request for the exercise of the rights established in Articles 15 to 22 of Regulation (EU) 2016/679 of the European Parliament and of the Council, of April 27, 2016, it shall be initiated by an admission agreement, which shall be adopted in accordance with the provisions of Article 65 of this organic law.

In this case, the deadline for resolving the procedure shall be six months from the date on which the admission agreement was notified to the complainant. After this period, the interested party may consider their complaint to be upheld."

Article 58.2 of the GDPR grants the Spanish Agency for Data Protection a series of corrective powers to correct any infringement of the GDPR, including "ordering the controller or processor to address the requests for the exercise of the rights of the data subject under this Regulation."

III

Rights of Individuals Regarding Personal Data Protection

The rights of individuals regarding personal data protection are regulated in Articles 15 to 22 of the GDPR and Articles 13 to 18 of the LOPDGDD. These include the rights of access, rectification, erasure, objection, the right to restriction of processing, and the right to data portability.

The formal aspects related to the exercise of these rights are established in Articles 12 of the GDPR and 12 of the LOPDGDD.

Additionally, consideration is given to the statements made in Recitals 59 and following of the GDPR. In accordance with the provisions of these regulations, the data controller must establish means and mechanisms to facilitate the exercise of rights by the data subject, which shall be free of charge (without prejudice to the provisions of Articles 12.5 and 15.3 of the GDPR), and is obliged to respond to requests made no later than one month, unless they can demonstrate that they are unable to identify the data subject, and to express their reasons in case they do not intend to comply with such request. The burden of proof for compliance with the duty to respond to the request for the exercise of rights made by the affected party lies with the controller.

The communication directed to the data subject regarding their request must be expressed in a concise, transparent, intelligible, and easily accessible form, using clear and plain language.

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IV

Right of Access

In accordance with the provisions of Article 15 of the GDPR and Article 13 of the LOPDGDD, "the data subject has the right to obtain from the data controller confirmation as to whether or not personal data concerning them is being processed and, if so, the right of access to the personal data."

Regarding the right of access to personal data, according to the provisions of Article 13 of the LOPDGDD, when the exercise of the right refers to a large amount of data, the data controller may request the affected party to specify the "data or processing activities to which the request refers." The

right will be considered granted if the data controller provides remote access to the data, thus fulfilling the request (although the data subject may request the information referred to in the aspects provided for in Article 15 of the GDPR).

The exercise of this right may be considered repetitive if it is exercised more than once within a period of six months, unless there is a legitimate reason for it. On the other hand, the request will be considered excessive when the affected party chooses a means other than the one offered that incurs a disproportionate cost, which must be borne by the affected party.

Like the other rights of the data subject, the right of access is a personal right. It allows the citizen to obtain information about the processing of their data, the possibility of obtaining a copy of the personal data concerning them that is being processed, as well as information, in particular, about the purposes of the processing, the categories of personal data concerned, the recipients or categories of recipients to whom the data may be communicated, the expected retention period or criteria for retention, the possibility of exercising other rights, the right to lodge a complaint with the supervisory authority, the available information about the source of the data (if it has not been obtained directly from the data subject), the existence of automated decisions, including profiling, and information about transfers of personal data to a third country or to an international organization. The possibility of obtaining a copy of the personal data being processed will not adversely affect the rights and freedoms of others; that is, the right of access will be granted in such a way that it does not affect third-party data.

The right of access in relation to medical records is specifically regulated in Article 18 of Law 41/2002, of November 14, which is the basic regulation governing Patient Autonomy and Rights and Obligations in the Field of Information and Clinical Documentation (hereinafter LAP), the literal text of which states: "1. The patient has the right of access, with the reservations indicated in section 3 of this article, to the documentation of the medical record and to obtain a copy of the data contained therein. Health centers will regulate the procedure that guarantees the observance of these rights.

2. The patient's right of access to the medical record may also be exercised by duly accredited representation.

3. The patient's right of access to the documentation of the medical record cannot be exercised to the detriment of the right of third parties to the confidentiality of the data contained therein collected in the therapeutic interest of the patient, nor to the detriment of the right of the professionals involved in its preparation, who may oppose the right of access the confidentiality of their subjective notes.

4. Health centers and individual practitioners will only facilitate access to the medical records of deceased patients to persons linked to them for family reasons or de facto, unless the deceased expressly prohibited it and this is evidenced. In any case, access by a third party to the medical record motivated by a risk to their health will be limited to the relevant data. Information affecting the privacy of the deceased or the subjective notes of professionals, or that harms third parties, will not be provided."

In this regard, it is important to highlight Article 15 of the LPA, which outlines the minimum content of the medical record:

"1. The medical record will include the information deemed essential for the truthful and updated knowledge of the patient's health status. Every patient or user has the right to have a record, in writing or in the most appropriate technical support, of the information obtained in all their care processes, carried out by the health service both in primary care and specialized care.

2. The main purpose of the medical record will be to facilitate healthcare, documenting all those data that, in the medical opinion, allow for the truthful and updated knowledge of the health status.

The minimum content of the medical record will be as follows:

- a) Documentation related to the clinical-statistical sheet.
- b) Admission authorization.
- c) Emergency report.
- d) Anamnesis and physical examination.
- e) Progress notes.
- f) Medical orders.
- g) Consultation sheet.
- h) Reports of complementary examinations.

- i) Informed consent.
- j) Anesthesia report.
- k) Operating room report or delivery record.
- l) Pathological anatomy report.
- m) Nursing care progress and planning.
- n) Therapeutic nursing application.
- ñ) Vital signs chart.
- o) Clinical discharge report.

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Paragraphs b), c), i), j), k), l), ñ), and o) will only be enforceable in the completion of the medical record when it concerns hospitalization processes or as otherwise provided.

3. The completion of the medical record, in aspects related to direct patient care, will be the responsibility of the professionals involved.

4. The medical record will be maintained with criteria of unity and integration, at each healthcare institution at a minimum, to facilitate the best and most timely knowledge by the healthcare providers of the data of a particular patient in each care process.

Regarding the preservation of the medical record, Article 17 of the LPA, in its points 1 and 5, states that:

"1. Healthcare centers have the obligation to preserve clinical documentation under conditions that guarantee its proper maintenance and security, although not necessarily in the original format, for the proper care of the patient for the appropriate time in each case and, at a minimum, five years from the date of discharge from each care process (...)

5. Healthcare professionals who carry out their activity individually are responsible for the management and custody of the care documentation they generate."

V

Conclusion

In the present case, from the examination of the documentation provided by the parties during the processing of this procedure, it has not been established that the responding party has facilitated, or expressly denied, the right of access, especially regarding the photographs and dental molds.

Thus, it cannot be accepted that the response that corresponds to be given can manifest itself on the occasion of a mere administrative procedure, such as the formulation of allegations due to the present procedure, initiated precisely for not properly addressing the request in question.

The aforementioned rules do not allow for the request to be overlooked as if it had not been raised, leaving it without the response that those responsible must issue, even in the event that there are no data in the files or even in those cases where it does not meet the stipulated requirements, in which case the recipient of such request is also obliged to require the correction of the observed deficiencies or, if applicable, to deny the request with justification indicating the reasons why the right in question cannot be considered.

Therefore, the request made obliges the responsible party to provide an express response in all cases, using any means that justifies the receipt of the response.

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Since a copy of the necessary communication that must be sent to the requesting party informing them about the decision made regarding the exercise of rights request is not attached, it is appropriate to

uphold the complaint that originated this procedure.

On another note, the health regulations that govern the medical record, as previously stated, establish its minimum content. If, as the requesting party states, in this case, the medical record is incomplete due to not having filled in some of the details, they have the possibility of addressing the corresponding health authorities to file their complaint, as this Spanish Agency for Data Protection does not have the competence to analyze and evaluate the content included by healthcare personnel.

In light of the cited provisions and others of general application, the Director of the Spanish Agency for Data Protection RESOLVES:

FIRST: TO UPHOLD the complaint filed by Ms. A.A.A. considering that the provisions of Article 15 of the GDPR have been infringed and to urge CLÍNICA TORCALDENT, S.L. with NIF B93597755, to send to the requesting party, within ten working days following the notification of this resolution, a certification addressing the requested right or denying it with justification indicating the reasons why it is not appropriate to address the request regarding the photographs and mold requested, in accordance with the provisions established in the body of this resolution.

The actions taken as a result of this Resolution must be communicated to this Agency within the same timeframe. Non-compliance with this resolution could result in the commission of an infringement of Article 83.6 of the GDPR, classified as very serious for the purposes of prescription in Article 72.1.m) of the LOPDGDD, which will be sanctioned in accordance with Article 58.2 of the GDPR.

SECOND: NOTIFY this resolution to Ms. A.A.A. and to CLÍNICA TORCALDENT, S.L.

In accordance with the provisions of Article 50 of the LOPDGDD, this Resolution will be made public once it has been notified to the interested parties.

Against this resolution, which concludes the administrative procedure in accordance with Article 48.6 of the LOPDGDD, and in accordance with the provisions of Article 123 of the LPACAP, the interested parties may optionally file an appeal for reconsideration before the Director of the Spanish Agency for Data Protection within one month from the day following the notification of this resolution or directly file a contentious-administrative appeal before the Contentious-Administrative Chamber of the National Court, in accordance with the provisions of Article 25 and in section 5 of the fourth additional provision of Law 29/1998, of July 13, regulating the Contentious-Administrative Jurisdiction, within two months from the day following the notification of this act, as provided in Article 46.1 of the aforementioned Law.

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